

Public Health Emergency Operations Center

"COUSP DRC"

MVE B-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°079/MVE B_01/08/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (49)	• Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (4/23): Kabondo, Lubunga, Makiso-Kisangani, Mangobo
Reporting date	August 1, 2026
Publication date	August 2, 2026

INDICATORS KEYS — CURRENT TO JULY 31, 2026

CONFIRMED CASES — 5 PROVINCES 3,748 +74 new cases (Ituri 49 · N-Kivu 22 · Haut-Uélé 3) 	CONFIRMED DEATHS · LEATHITIS 1,657 (44.2%) +36 deaths (Ituri 23, North Kivu 11, Haut-Kivu 11) Uele 2) 	SUSPECTED CASES OF THE DAY 227 Ituri 153 · N-Kivu 66 · Haut-Uélé 4 · South Kivu 4 
PATIENTS IN ISOLATION 690 Ituri 476 · N-Kivu 181 · Haut-Uélé 31 · Tshopo 2 	CURED — CUMULATIVE 708 +42 recovered (Ituri 83 · North Kivu 4) 	NATIONAL CONTACT TRACKING 80.1% 13,988 / 17,472 views · Ituri 83.4% · N-Kivu 75.1% · H-Uélé 66.7% · Tshopo 66.2% · South Kivu 100% 

1. Epidemiological Highlights

- **Epidemiological situation:** No new health zones affected in the last 24 hours. The total remains at 49 affected health zones out of 140 across the five provinces. Tshopo province reports a fifth affected zone, Wanie-Rukula, where the 7th provincial case was confirmed on July 29; this classification is not included in the cumulative totals pending harmonization with the provincial health division.
- **Case trends:** In the last 24 hours, 74 new confirmed cases, 36 confirmed deaths and 42 recoveries were recorded.

In **Ituri**, 49 new confirmed cases were reported in Rwampara (11), Nizi (10), Bunia (9), Lita (5), Mongbwalu (5), Mangala (3), Nia-Nia (2), Damas (1), Drodoro (1), Komanda (1), and Mambasa (1), along with 23 deaths, 13 of which were community deaths (56.5%). Eighteen (18) positive retests were also returned, bringing the total number of positive results for the day to 67.

North **Kivu**: 22 new confirmed cases were reported in Katwa (12), Butembo (5), Beni (4), and Musienene (1), along with 11 deaths, including 7 community deaths. The provincial total reaches 400 confirmed cases and 262 deaths, representing a case fatality rate of 65.5%.

In **Haut-Uélé**, three new confirmed cases were reported (two in Pawa and one in Wamba), with the fourth positive result of the day being a repeat test in Wamba. Two deaths were confirmed at the Pawa treatment center. The provincial total now stands at 60 confirmed cases and 30 deaths, representing a case fatality rate of 50.0%, spread across 5 of the 13 health zones.

o **Tshopo** : no new confirmed cases or deaths have been reported. The provincial total remains at 7 confirmed cases.

5 deaths and 1 recovery, representing a case fatality rate of 71.4%.

South **Kivu** : The provincial total remains at 3 confirmed cases, including 1 death and 2 recoveries. Contact tracing has resumed: 78 contacts listed, 23 discharged, and 55 still being monitored, all seen (100%), including 41 in quarantine; note **that these are contacts of a confirmed death imported from Tshopo province**. The number of days without a new confirmed case in Miti-Murhesa is 52 days since June 10th, while the actual interval is 51 days.

- **Analysis:** Ituri remains the epicenter of the epidemic, accounting for 87.5% of cumulative cases and 66.2% of new daily confirmations. Contact tracing has improved to 80.1%, compared to 78.0% the previous day, but remains below the operational threshold of 95%, while the isolation ward has decreased to 690 patients, for an overall occupancy rate of 65.7%. The alert investigation rate has risen to 87.2%, although the volume of alerts has been reduced, as Haut-Uélé did not submit any alert management slides.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it

The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 49 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.

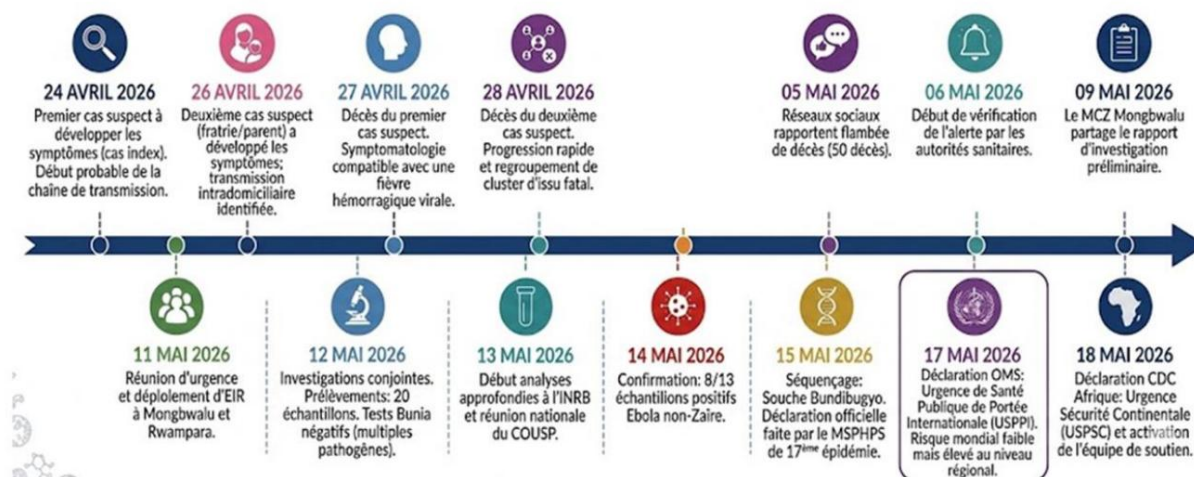


Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Ituri	3,278	1,359	41.5%	28 / 36 (77.8%)	49
North Kivu	400	262	65.5%	11/34 (32.4%)	22
Haut-Uélé	60	30	50.0%	5/13 (38.5%)	3
Tshopo	7	5	71.4%	5/23 (21.7%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,748	1,657	44.2%	50 out of 140	74

Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 87.5% of confirmed cases (n = 3,278) and 82.0% of deaths (n = 1,359), resulting in a case fatality rate of 41.5%. The health zones of Bunia (905 cases), Rwampara (644), Mongbwalu (550), Nizi (397), Lita (137), and Nyankunde (115) remain the most affected, accounting for 83.8% of reported cases at the provincial level and 73.3% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	3,278	1,359	41.5%	49	13	10	23
Adja	11	1	9.1%	0	0	0	0
Aru	5	5	100.0%	0	0	0	0
Ariwara	7	2	28.6%	0	0	0	0
Aungba	9	5	55.6%	0	0	0	0
Bamboo	60	13	21.7%	0	0	1	1
Boga	1	1	100.0%	0	0	0	0
Bunia	905	262	29.0%	9	2	1	3
Damascus	18	8	44.4%	1	0	0	0
Drodro	11	6	54.5%	1	0	0	0
Fataki	35	17	48.6%	0	0	0	0
Gethy	1	0	0.0%	0	0	0	0
Kambala	2	0	0.0%	0	0	0	0
Kilo	29	11	37.9%	0	0	0	0
Komanda	37	27	73.0%	1	1	1	2
Lita	137	80	58.4%	5	0	1	1
Logo	9	3	33.3%	0	0	0	0
Lolwa	7	2	28.6%	0	0	0	0
Mahagi	4	1	25.0%	0	0	0	0
Mambasa	10	6	60.0%	1	0	0	0
Mandima	16	12	75.0%	0	0	0	0
Mangala	105	66	62.9%	3	2	0	2
Mongbwalu	550	261	47.5%	5	2	0	2
Nia-Nia	103	56	54.4%	2	1	2	3
Nizi	397	192	48.4%	10	3	2	5
Nyankunde	115	34	29.6%	0	0	2	2
Rimba	9	4	44.4%	0	0	0	0
Rwampara	644	260	40.4%	11	2	0	2
Tchomia	41	24	58.5%	0	0	0	0

North Kivu	400	262	65.5%	22	7	4	11
Blessed	61	49	80.3%	4	1	2	3
Butembo	88	67	76.1%	5	1	1	2
Goma	1	0	0.0%	0	0	0	0
Kalunguta	6	3	50.0%	0	0	0	0
Katwa	180	108	60.0%	12	5	1	6
Kyondo	13	8	61.5%	0	0	0	0
Mabalako	1	0	0.0%	0	0	0	0
Masereka	4	0	0.0%	0	0	0	0
Musienene	41	23	56.1%	1	0	0	0
Oicha	4	3	75.0%	0	0	0	0
Vuhovi	1	1	100.0%	0	0	0	0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%	0	0	0	0
Haut-Uélé	60	30	50.0%	3	0	2	2
Boma	12	4	33.3%	0	0	0	0
Mangbetu	12	4	33.3%	0	0	0	0
Isiro	19	9	47.4%	0	0	0	0
Pawa	12	9	75.0%	2	0	2	2
Rungu	1	0	0.0%	0	0	0	0
Wamba	16	8	50.0%	0	0	0	0
Tshopo	7	5	71.4%	10	0	0	0
Kabondo	1	1	100.0%	0	0	0	0
Lubunga	1	0	0.0%	0	0	0	0
Makiso-Kisangani	3	3	100.0%	0	0	0	0
Mangobo	1	1	100.0%	0	0	0	0
Wanie-Rukula	1	0	0.0%	0	0	0	0
Total	3,748	1,657	44.2%	74	20	16	36

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalu, Nizi, Lita and Nyankunde, with transmission continuing in the areas already affected without any new geographical expansion in the last 24 hours.

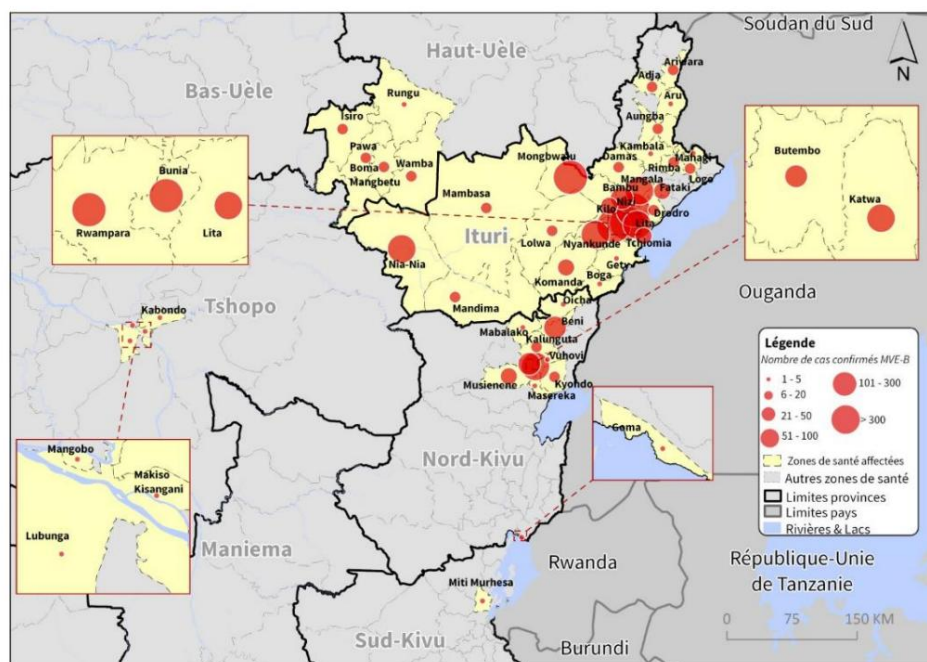


FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

Transmission in already affected health zones underscores the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S26 to S31, with fluctuations related to notification delays and the gradual consolidation of data.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

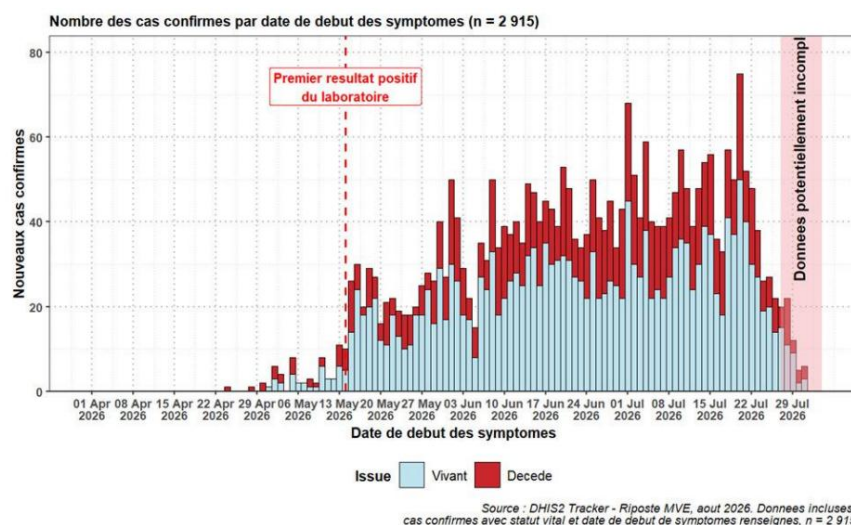


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

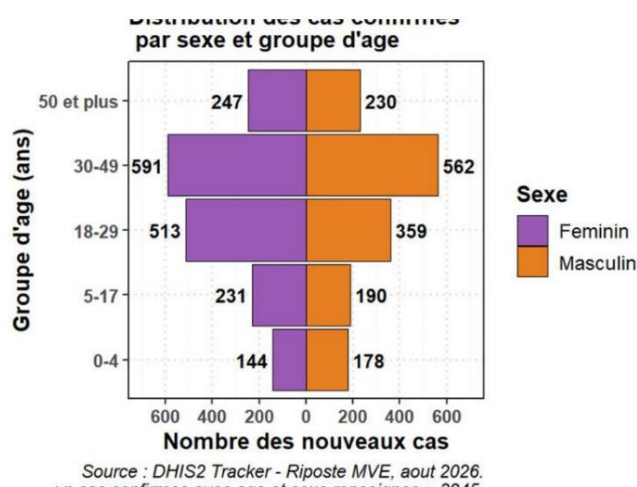
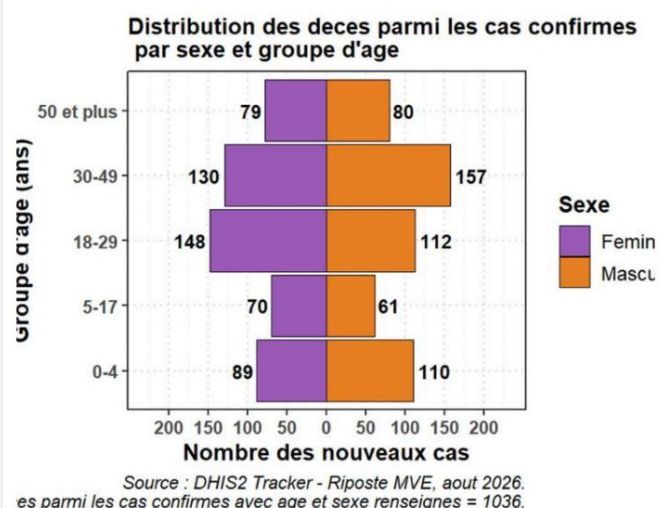


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	10	65	ND	0	80
New alerts received	479	571	ND	20	1077
Total alerts for the day	489	636	ND	12	1,157
Alerts investigated	421	571	ND	12	1009
Investigation rate (24 h)	86.1%	89.8%	ND	100.0%	87.2%
Alerts confirmed — live	101	42	ND	4	150
Alerts confirmed — deceased (comm.)	52	24	ND	0	77
Total suspected cases for today	153	66	ND	4	227

In total, 1,157 alerts were managed (including 1,077 new ones), 1,009 were investigated within 24 hours (87.2%), and 227 were validated (150 active cases and 77 community deaths). North Kivu accounts for the majority of the volume with 571 alerts investigated at 89.8%, while Ituri is at 86.1% with 489 alerts, and South Kivu investigated all 12 of its alerts. Tshopo investigated only 5 out of 20 (25.0%), based on data collected up to July 30. Haut-Uélé does not include any alert management slides in its August 1st report: its indicators are marked ND, and the national total therefore only covers four provinces.

3.2 Laboratory: tests and positivity

ŷ **Ituri** : 290 samples documented in the collection network, including 67 positive (36 live and 13 dead), — 49 new cases confirmed and 18 re-samples, representing an apparent positivity of 23.1%.

ŷ **North Kivu**: 153 samples received, of which 22 were positive (Beni 18, Butembo 4), representing a positivity rate of 14.4%; 4 analyses remain in progress.

ŷ **Haut-Uélé**: 9 samples received (Isiro 6, Pawa 2 and Rungu 1) and 19 analyzed, including remnants: 4 positive results (Wamba 2 and Pawa 2), including one re-sample.

ŷ **Tshopo**: no samples were reported for August 1st; the provincial total stands at 7 positive cases out of 51 tests. Four samples had been taken from 20 alerts on July 30.

ŷ **South Kivu**: 5 samples received, none positive. The provincial total stands at 3 positives out of 295 samples analyzed (1.0 %). The Miti-Murhesa health zone remains the only one affected in the province.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri*	10,848	9050	83.4%
North Kivu*	5,942	4,465	75.1%
Tshopo**	65	43	66.2%
Haut-Uélé***	562	375	66.7%
South Kivu	55	55	100.0%
Total	17,472	13,988	80.1%

* In Ituri and North Kivu, the follow-up rate was calculated only for health zones that reported. The health zones of Kambala, Damas, and Mandima (Ituri) and Rungu (Haut-Uélé) did not have usable data at the time of report consolidation. Tshopo did not report its contact tracing for July 31.

Contact tracing remains insufficient at the national level (80.1%), below the operational threshold of 95%, even though it has improved by 2.1 percentage points compared to the previous day. Ituri has followed up 9,050 contacts out of 10,848 (83.4%), with the Damas, Logo, and Kambala areas reporting no results; the Mangala area remains the most affected, with 493 contacts seen out of 1,565 (31.5%). North Kivu has 4,465 contacts seen out of 5,942 (75.1%) and has 239 contacts lost to follow-up. Haut-Uélé has 375 contacts seen out of 562 (66.7%), figures identical to those of the previous day. Tshopo reports 43 contacts seen out of 65 (66.2%) as of July 30 and South Kivu has seen all 55 of its contacts (100%).

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 0 alerts notified

No alerts have been issued to crossing points in the province in the last 24 hours.

North Kivu — 2 alerts notified

Two alerts were notified: a lifeless body intercepted at the PK5 PoC — a 43-year-old man traveling from Kasindi to Beni — directed to the morgue for swab (100%), and a live alert at the Mukulia PoC, invalidated after investigation.

Tshopo — not reported

No PoE/PoC slides are included in the July 31st deck: the province's indicators are marked ND in Table 5, at: 4 PoE/PoC operational out of 19 (21.1%), 1,400 travellers checked, all screened and sensitized, 99.7% having washed their hands, and no alerts notified. The PoCs at PK 23/RN4 and PK 23/RN3 remained unreachable, with reporting completeness set at 50%.

Haut-Uélé — 0 alerts notified

No alerts were notified to the province's activated PoE/PoC devices.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri North Kivu		South Kivu	Tshopo	High-Uele	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (16/16)	100% (9/9)	21.1% (4/19)	30.3% (10/33)	72.3%
Number of people who have switched to PoE/PoC	103,673	63,731	83,848	1,400	13,507	266,159
% of travelers screened % of	97.1%	96.8%	100.0%	100.0%	100.0%	98.1%
travelers who washed their hands	96.8%	96.8%	100.0%	99.8%	100.0%	98.0%
% of travelers made aware	99.1%	96.8%	100.0%	100.0%	100.0%	98.9%
Number of alerts notified	0	2	0	0	0	2
Number of validated live alerts	0	0	0	0	0	0
Number of bodies recovered today	0	1	0	0	0	1
Number of problematic contacts intercepted today	0	0	0	0	0	0
% of validated live alerts referred to CT/CTE	0	0	0	0	0	0
% of lifeless bodies swabed	0	100% (1/1)	0	0	0	100% (1/1)
Number of positive results of referred suspected cases	0	0	0	0	0	0

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator	Ituri	North Kivu	Haut-Uele	Tshopo	Together
Number of beds	821	141	68	20	1,050
Patients in bed (Day -1)	525	181	33	1	740
Total admissions (24 hours)	61	44	16	1	122
Total admissions	905	412	63	3	1,383
Outings					
Deceased	17	4	4	0	18
No case	51	35	0	0	71
Healed	38	4	0	0	15
Escapees	4	0	0	0	2
Transferred to the HGR	0	1	0	0	0
Total outflows (24 h)	110	44	4	0	158
Patients in isolation (end of day),	476	181	31	2	690
including confirmed cases	296	23	13	1	333
including suspects	180	158	18	1	357
Occupancy rate	58.0%	128.4%	45.6%	10.0%	65.7%

In total, 690 patients were in isolation, including 333 confirmed cases and 357 suspected cases, for an overall occupancy rate of 65.7% (690 patients for 1,050 beds), with 122 admissions and 158 discharges (42 recovered, 25 deaths, 86 non-cases, 4 escaped and 1 transferred).

Ituri bears the brunt of the burden with 476 patients in isolation (296 confirmed, 180 suspected) out of 821 beds, representing 58.0% occupancy; its indicator slide reverses the numbers of confirmed and suspected cases, with the breakdown by site being the determining factor.

North Kivu, 181 patients are in isolation (23 confirmed, 158 suspected) for an occupancy rate of 128.4% (141 beds, capacity not reported for six cycles). Haut-Uélé has 31 patients in isolation (13 confirmed, 18 suspected) for 68 beds distributed across five treatment centers (45.6%), with the Wamba treatment center exceeding its capacity for confirmed cases (125%); its daily patient flow is not complete — 45 patients expected versus 31 reported, including 9 suspected cases discharged without justification from Boma Mangbetu. Tshopo has 2 patients in isolation for 20 beds (10.0%). In Ituri, the report indicates six facilities are saturated for confirmed cases but only lists five — Rwampara, CME, ISTM, Lita, and Fataki — and two for suspected cases (Aru and Fataki). In terms of overall occupancy, the Nizi (144%), Fataki (125%), and Aru (112%) CTEs are exceeding their capacity.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	22 (84.6%)	ND
Confirmed cases are being monitored.	160 (78.8%)	ND
New suspected cases	27 (77.1%)	ND
Suspected cases are being monitored	119 (100.0%)	ND
Laboratory results announced (including positive ones)	71 (22)	ND
Children separated — nursery / community	0/45	ND
CTE support staff	258	ND
Frontline Personnel (PPL)	38	ND
Members of households and communities	44	ND
EDS supported (SMSPS)	12	ND

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE WORKERS (FWs) INFECTED WITH EVD IN ITURI

Health Zone	Confirmed cases	Healed	In hospitalization	Death
Bunia	35	19	1	14
Rwampara	33	25	1	7
Mongbwalu	21	10	2	9
Nyankunde	14	11	1	2
Nia-Nia	20	1	13	4
Lita	4	1	2	1
Nizi	4	0	1	3
Mangala	4	2	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Logo	3	0	3	0
Total — case fatality rate 28.9%	142	72	26	41

PPL = healthcare personnel and frontline providers.

In Ituri, 142 cases of PPL (Patients of Life-Throughput Disease) have been confirmed, including 41 deaths, representing a case fatality rate of 28.9%. A new case was confirmed today in the Nizi health zone. The breakdown by outcome shows only 139 PPL cases (72 recovered, 26 hospitalized, and 41 deaths): three PPL cases.

These cases remain undistributed and must be allocated by the provincial health division. The IPC pillar, for its part, shows a cumulative total of 134 infected PPLs out of 2,884 exposed individuals (4.6%), a discrepancy that needs to be reconciled with the surveillance register. In North Kivu, 11 PPLs are confirmed, including 3 deaths; Haut-Uélé reports 3, representing 5.0% of its confirmed cases, one of whom is hospitalized at the Isiro Ebola Treatment Center.

7 Pillar-Based Response Activities

COORDINATION Ÿ The coordination meeting for the response was held, and the activities of the pillars were monitored. In Tshopo, the SGI meeting adopted three recommendations with a deadline of August 3: harmonize the data from the surveillance and laboratory pillars, which diverge on several cases; conduct advocacy with a religious leader in Kisangani who is spreading messages denying the existence of Ebola virus disease; and organize a coordinated response to the publication of photos of patients and corpses on social media.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) Ÿ Ituri: 163 health centers have been pre-selected in 137 health areas across 36 zones, including 92 state-run, 48 faith-based, and 23 private centers; 43 of these already receive support from a partner. Costing establishes a monthly need of USD 2,209,095 to USD 3,976,371 (77 primary care centers for USD 1,842,329 and 86 primary care centers for USD 366,766). The official directive prohibiting any payments at the targeted health facilities and the contracting with stakeholders remain to be implemented. Phase 2, "health security," includes triage at the entrance, a separate circuit for suspected cases, and rapid SOP/MEGS/ PCI training.
LOGISTICS Ÿ Ituri: Ensuring the mobility of different pillars of the response. Ÿ North Kivu: 50 operational motorcycles out of 100 expected, 6 vehicles out of 7, 5 ambulances out of 30, no hearses out of 20, 2 CTEs built according to standards out of 11, and no CTs out of 36. No area is free from shortages of IPC inputs, and only 4 out of 11 areas are Supplied with inputs for holistic care; 8 out of 11 zones have submitted their data. The July inventory continues, and support is being provided to the Beni health zone.	PCI / WASH Ÿ Ituri: 8 out of 29 health zones (27.5%). 27 rings were opened around cases and 36 sites identified, all decontaminated with verified proof, representing a 100% completion rate and no discrepancies beyond 48 hours. 34 out of 98 case households were decontaminated within 48 hours (34.6%), in Nizi (16), Bunia (8), Mongbwalu (6), and Lita (4); 4 health facilities were decontaminated (Rwampara 2, Bunia 1, Lita 1) as well as 3 means of transport (Lita 2, Komanda 1). 35 health facilities were monitored and supported out of a target of 394 (Bunia 23, Rwampara 10, Mambasa 2), and 46 service providers were trained (Komanda 26, Bunia 20). The pillar received 10 tablets, a laptop, a printer and an overhead projector). Ÿ North Kivu: One COVID-19 treatment center (DTC) was located in Musienene, and three DTCs in Beni received incomplete infection prevention and control (IPC) kits. A scorecard assessment was conducted at one DTC in Musienene, with an average score of 70.9% in the Nduko health area. Follow-up and support were provided to 44 DTCs (24 in Oicha, 20 in Beni), and 193 healthcare providers were briefed during formative supervision sessions (93 in Oicha, 88 in Beni, and 12 in Musienene). Only 4 out of 11 areas achieved completeness of the IPC kits.
COMMUNICATION (RCCE / CREC) Ÿ Ituri: 3,436 people reached during 47 home visits, 3,372 during 22 educational talks and focus groups, 5,705 during 25 mass awareness campaigns, and 10 people engaged following 2 advocacy sessions, mainly in the Nyankunde and areas of	HOLISTIC CARE (HCC) Ÿ Ituri: 38 new recoveries; 476 patients in isolation (296 confirmed, 180 suspected), 61 new admissions and 110 discharges, including 38 recoveries, 17 deaths, 51 non-cases, and 4 escapees. The cumulative number of recoveries reaches 649. Pillar meeting, transfer of patients from ESS, CT, and community to the

Rwampara. 52 community alerts were reported (50 living and 2 dead), all in Nyankunde. Resistance was observed and not overcome, linked to delays in the EDS teams and household decontamination; 65 feedback points were identified, 40 were captured, and 25 were clarified, representing 38%. Capacity building: 42 decontamination workers in Nia-Nia, Mambasa, and Mandima (WHO), 115 community health workers in Rwampara (UNICEF/REMED), 15 team members in Komanda (Save the Children), and 30 motorcyclists in Tchomia (FHI 360). Two radio programs produced, six public service announcements broadcast, 36 community radio stations engaged, and 15 IEC materials distributed. The commander of the 4th FPIC brigade pledged his support in Rwampara for the response activities. ŷ North Kivu: Community dialogue in the Butsili neighborhood of Beni, bringing together 305 community leaders and members; briefing of 62 CAC members in Beni, 40 service providers and traditional healers in Musienene, 35 writing club facilitators in Vuhovi, and 29 small livestock vendors at the Kyondo market. 7,569 households visited out of 12,671 (59.7%), 25,111 people reached through door-to-door visits (90.9%), 2,588 in public places (39.8%), and 1,252 during dialogues and discussions (56.9%); 754 people engaged, 180 alerts of suspected cases reported by community actors out of 450 expected (40%), and all 34 targeted radio stations broadcast. Two incidents were resolved within 48 hours, including the ransacking of an ambulance at the La Joie medical center, Cité Belge district in Beni. ŷ Tshopo: Continued mass awareness campaigns and 63 during talks; 3 of the 14 targeted radio stations broadcast; start of translation of IEC tools into Mayogo and Mangbetu.	CTE, announcement of results and monitoring of CTE expansion and construction work; continuation of training for CTE service providers in Rwankole and monitoring of PECH data entry in DHIS2 at the Bunia General Referral Hospital CTE. UNICEF, through the PPSSP, began water drilling at the CME. ŷ North Kivu: 155 confirmed cases admitted to Ebola Treatment Centers (ETCs) with 55 deaths, representing an ETC mortality rate of 35%, and 16 lost to follow-up. There were 44 admissions and 44 discharges today (4 recovered, 4 deceased, 35 non-cases, and 1 transfer), leaving 181 patients in isolation at the end of the day (23 confirmed, 158 suspected). Regarding nutrition, 7 confirmed unstable patients received specific diets, 25 confirmed and 45 suspected cases received hot meals, as did 76 frontline healthcare providers and 7 caregivers; 5 children aged 0-59 months and pregnant or breastfeeding women with acute malnutrition were treated, and one separated infant is receiving breast milk substitute in Kalunguta. Briefing of service providers by PRONANUT with the support of APETAMACO/PAM, induction of management teams from Katwa and Butembo and evaluation mission to Musienene. ŷ Haut-Uélé and Tshopo: 2 confirmed cases have been admitted (Boma Mangbetu 1, Wamba 1) and 5 suspected cases (Wamba 3, Pawa 2), for a total of 37 patients hospitalized at the end of the day (16 confirmed, 21 suspected), with admissions and discharges for the day not reported; the Wamba CTE exceeds its
RESEARCH, INNOVATION & VACCINATION ŷ Ituri: Visit by a WHO delegation to the office of the pillar leader at the DPS, receipt of results from the Partners and EBO-PEP clinical trials, and preparation of teams for the opening of activities in the Nizi health zone with the support of the WHO — pillar meeting, meeting with the area management team and visit to the training room. The total number of participants included and followed was 22, of which 19 were on the product or placebo and 3 were on RMD. Outlook: Continue receiving partner protocols for analysis and certification, and facilitate the execution of research certified by national authorization. No vaccination activities are planned.	Dignified and secure burials (DSM) ŷ Ituri: 71 death alerts in total, 48 EDS (Emergency Health Data) tests carried out and 34 swabs (swabs) performed, including 17 in Ebola Treatment Centers (ETCs); the proportion of reporting zones was 13 out of 32, or 40.6%. Since May 20, the cumulative total has reached 3,247 alerts for 2,629 EDS tests carried out. Second day of training for the two EDS teams in the Tchomia Health Zone and support from the CREC (Regional Center for Ebola Prevention) pillar in raising awareness among 50 pastors of different religious denominations. Two refusals related to disbelief in Ebola Virus Disease (EVD) were recorded in Mandima; resistance persists in Mambasa, Nyankunde, Mandima, Aru, and Bambu.

reported by the provinces this jo the university community of Bunia and continued reception of protocols from partners for analysis and certification. No vaccination activity is reported by the provinces today.	ŷ North Kivu: 33 death alerts received, 33 bodies swabed and secured, and 31 DHS (Death Health Survey) carried out, without any failures; 16 swabed and secured bodies remain pending. Deaths occurring in the community constitute the majority of alerts, primarily in Beni (14) and Katwa (12). 34 community deaths benefited from a DHS and 43 were swabed.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ŷ Ituri: continuation of briefing and awareness-raising by the focal points of the Fataki and Mandima health zones, and continuation of the cascade briefing in the Ariwara health zone with the support of the WHO.	

8 Main challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT facilities: 128.4% occupancy in North Kivu (capacity of 141 beds: not reported for six cycles) and five facilities beyond or at the limit of their capacity for confirmed cases in Ituri, including the Nizi CTE at 278% and ISTM CTE at 110%;	Admission delays, increased risk of nosocomial transmission Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Insufficient contact tracing performance: Ituri 83.4%, North Kivu 75.0% and South Kivu 100%, the Haut-Since Uélé and Tshopo did not provide data; the health zones of Damas, Logo and Kambala (Ituri) remained without usable data	Non-transmission chains interrupted Train, deploy and motivate community liaisons (RECOs); strengthen supervision; consolidate the recovery observed in Ituri
5	Ambulance shortages and pre-positioning at points of control	Delays in transfer and isolation Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care Advocacy and urgent supply of essential medicines

Challenge No.		Impact	Actions required
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (181/1170, i.e. 15%)	Risk of nosocomial infections (141 cases of PPL, including 41 deaths in Ituri; 11 PPL, including 3 deaths in North Kivu and 3 PPL confirmed at Haut-Uélé)	Urgent supply, strengthening of IPC training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Rwampara, Fataki, Bambu-mine, Boga and Mambasa)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Inadequacy of the financial resources	Limitation of the implementation of pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities

Briefing of IT and ITA staff on PECH in the context of Ebola Virus Disease in Tshopo Province





Exchange between the WHO delegation (Geneva) and the research pillar team: RESEARCH, innovation and vaccination

FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
CONTACT**

For the National Institute of Public Health (INSP) of the DRC

The Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: diedonne.mwamba@insp.cd

Tel.: +243 816 040 145

The COU-SP Coordinator

Prof. NGANDU Christian

Email: Christian.ngandu@insp.cd

Tel.: +243998091915

The Incident Manager

Prof. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

With technical support from the World Health Organization (WHO) and
Africa CDC